

GULFSHORE VASCULAR & ENDOVASCULAR

412 Bayshore Boulevard, Suite 300 | Tampa, FL 33606 | (813) 555-0140 | Fax (813) 555-0141
NPI: 1497753102 | Tax ID: 59-5550142 | Fictional practice created for demonstration

PATIENT MEDICAL CHART

CONFIDENTIAL — FOR AUTHORIZED USE ONLY. All names, addresses, member IDs, and clinical facts are fictional and were created for demonstration. This is not a real patient.

1. Patient demographics

Patient name	Samuel Reed
Date of birth	July 30, 1954
Age at visit	71 years
Sex	Male
MRN	PAD-34102
Address	908 South Howard Avenue, Tampa, FL 33606
Phone	(813) 555-0148
Emergency contact	Ruth Reed (spouse), (813) 555-0149
Primary care	Dr. Mark Ellison, MD, (813) 555-0160
Preferred language	English

2. Insurance

Primary insurance

Payer	Palm Mutual of Florida, Medicare Advantage
Member ID	PMMA771205
Group	PM-MA-09
Prior authorization	Required for peripheral stent placement
Auth telephone	1-800-555-0177

Secondary insurance. None on file.

3. Active problem list

ICD-10	Diagnosis	Onset	Status
I70.221	Atherosclerosis of native arteries of extremities with rest pain, right leg	2026	Active
I70.233	Atherosclerosis of native arteries of right leg with ulceration of ankle	2026	Active
E11.9	Type 2 diabetes mellitus without complications	2009	Active
Z87.891	Personal history of nicotine dependence	Quit 2019	Historical

4. Current medications

Medication	Dose	Frequency	Indication
Aspirin	81 mg	Daily	Antiplatelet
Rosuvastatin	20 mg	Daily	Lipids

Medication	Dose	Frequency	Indication
Metformin	1000 mg	Twice daily	Diabetes
Amlodipine	5 mg	Daily	Hypertension

He does not smoke. A structured walking program was not prescribed because of rest pain and a foot ulcer.

5. Allergies

Allergen	Reaction	Severity
Codeine	Nausea	Mild

6. Visit notes — vascular surgery

Visit date: March 2, 2026 | **Attending:** Priya Raman, MD, FSVS | **Type:** New patient consult

Chief complaint. Right foot rest pain and a nonhealing ulcer on the right great toe.

History of present illness. Samuel Reed is a 71-year-old retired bus driver. For six weeks he has woken at night with pain across the right forefoot that eases when he hangs the foot over the bed. That is ischemic rest pain. A 1.2 cm ulcer on the tip of the right great toe has been open for eight weeks and has not healed with dressing changes and offloading. He has no fever and no exposed bone. He quit smoking in 2019 and does not smoke now. He cannot join a walking program because rest pain and the open ulcer stop him after a few steps. He already takes aspirin.

Examination.

Blood pressure 146/80 mmHg	Right foot Dependent rubor, pallor on elevation. 1.2 cm dry ulcer, tip of the right great toe. No gangrene of other toes.
Pulses Right femoral present. Right pedal pulse absent. Left pedal pulse present.	Smoking Quit in 2019. Not a current smoker.
Infection No cellulitis and no purulence	

Assessment and plan.

Critical limb ischemia of the right leg: ischemic rest pain plus a nonhealing ischemic ulcer of the right great toe, attributed to the right superficial femoral lesion on CTA.

A 12-week exercise trial is not appropriate and was not started. The indication is critical limb ischemia, not claudication.

Order right femoropopliteal stent. Do not order lithotripsy.

Electronically signed: Priya Raman, MD, FSVS — March 2, 2026 at 8:20 AM

7. Diagnostic reports

Ankle-brachial and toe-brachial index — February 18, 2026

Ordering physician	Priya Raman, MD, FSVS
Right ABI	0.41
Right TBI	0.28
Left ABI	0.94

Interpretation. Right ankle-brachial index is 0.41, which is below 0.90. Right toe-brachial index is 0.28, which is below 0.70. The left ankle-brachial index is 0.94.

Electronically signed: Priya Raman, MD, FSVS — February 18, 2026

CTA runoff — February 10, 2026

Ordering physician	Mark Ellison, MD
---------------------------	------------------

Facility	Bayshore Imaging
Indication	Right foot rest pain and toe ulcer
Date	February 10, 2026, within six months of the March 2, 2026 request

Interpretation. Both common and external iliac arteries are patent. There is a 90 percent stenosis of the distal right superficial femoral artery over 8 cm, with a patent popliteal artery below it. Tibial vessels reconstitute. The left femoropopliteal segment has no stenosis over 50 percent. Impression: 90 percent stenosis of the distal right superficial femoral artery, consistent with the right foot ischemia. The iliac arteries are not the target.

Electronically signed: Owen Clarke, MD — February 10, 2026

8. Planned procedure

Planned procedure	Right femoropopliteal stent of the distal superficial femoral artery
CPT	37226 — femoral or popliteal revascularization with stent, unilateral
Laterality and vessel	Right superficial femoral artery
Not ordered	Intravascular lithotripsy, iliac stent, and bypass are not ordered
Surgeon	Priya Raman, MD, FSVS
Payer	Palm Mutual of Florida Medicare Advantage
Authorization status	Not submitted. Request dated March 2, 2026.

Clinical indication summary

Critical limb ischemia: ischemic rest pain of the right foot and a nonhealing ischemic ulcer of the right great toe for eight weeks.

Right ABI 0.41 and right TBI 0.28 on February 18, 2026.

CTA February 10, 2026: 90 percent stenosis of the distal right superficial femoral artery. Iliac arteries are patent.

Not a current smoker. No twelve-week walking trial was undertaken because the indication is critical limb ischemia rather than claudication.

Ordered code is 37226, a stent, not lithotripsy.

This document is for clinical and administrative demonstration only. Patient identifiers are fictional.